



Imaging Healthcare Specialists - Hillcrest
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Patient: MARTINEZ, LELIA M
Exam Date: 03/07/2024
Accession #: IHS3498707
At the SNOOK, BRIAN, DO
Request of: 1275 30TH STREET
SAN DIEGO, CA 92154

DOB: 11/09/1948
ID: 30000062756
Age: 75
Sex: Female
CC:

PROCEDURE:
DEXA BONE DENSITY STUDY

COMPARISON:
5/29/2020

INDICATIONS:
Patient has osteoporosis on X-ray, previous DXA exam.

TECHNIQUE:
Lumbar spine and proximal femoral dual-energy X-ray absorptiometry (DXA) was performed on a central Hologic device. In peri-menopausal women and in men age 50 or older, current International Society for Clinical Densitometry (ISCD) Official Positions state that the WHO Classification is determined by the lower T-score from either the Spine Analysis or from the Hip Analysis. The reported Hip Analysis T-score is based on either the femoral Neck or the Total proximal femur ROI whichever is lower. If bilateral hip measurements are obtained, Official Positions state that there is insufficient evidence to recommend mean readings. Reference: [Http://www.iscd.org/official-positions](http://www.iscd.org/official-positions).

HIP ANALYSIS:
The following results are obtained from the left neck femoral region which is the most abnormal ROI:
Bone mineral density (BMD) (g/cm²): 0.628
T-score (standard deviation relative to young adult mean BMD): -2.0
Z-score (standard deviation relative to age matched control group): 0.0

CHANGE SINCE PRIOR HIP EXAM:
Increased bone mineral density.

SPINE ANALYSIS:
Lumbar spine region of interest: L1 and L4. L2 and L3 were excluded secondary to discordance likely from hypertrophic changes.
Average bone mineral density (BMD) (g/cm²): 0.780
T-score (standard deviation relative to young adult mean BMD): -2.3
Z-score (standard deviation relative to age matched control group): 0.1

CHANGE SINCE PRIOR SPINE EXAM:
Increased bone mineral density.

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WHO CLASSIFICATION:

Osteopenia (T-score -1.0 to -2.5).

ADDITIONAL FINDINGS:

No significant additional findings.

FRACTURE RISK ASSESSMENT (FRAX) 10 YEAR FRACTURE PROBABILITY:

FRAX is a computer-based algorithm which uses easily obtained clinical risk factors combined with femoral neck BMD or femoral neck T-scores to estimate an individual 10-year fracture probability. FRAX with BMD predicts fracture risk better than clinical risk factors or BMD alone. It is not appropriate to use FRAX to monitor treatment response.

Major Osteoporotic Fractures: 7.4%

Hip Fracture: 1.8%

In general, FRAX-calculated risk of 20% or more for a major fracture and/or a FRAX-calculated risk of 3% or more for a hip fracture are considered indications for pharmacological treatment.

Dictated and Electronically Authenticated by: Borso, Maya, M.D. on 03/07/2024 08:57 AM Proofread by: Borso, Maya, M.D. on 03/07/2024 08:57 AM

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